



Request for Additional Security Staff Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	All areas	Medical Nursing Security

2. Purpose

Armadale Health Service has 24-hour security on site, however, there remains occasions where additional Security resources may be required on site for control of specific risks in the mental health.

This procedure aims to support a consistent approach in requesting additional (external) Security staff both within and after hours, and to ensure attention is paid to timely risk-related assessment, reviews, management strategies and resource allocation.

The values underpinning this procedure promote care that is recovery-oriented, person-centred, trauma-informed, culturally competent and developmentally appropriate.

This procedure should be read in conjunction with, [DOH Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#).

3. Procedure

A visual summary of this process can be located in [Section 3.4](#).

3.1 Identification and escalation of identified risk of imminent or serious risk.

1. Clinicians who identify heightened risk, must liaise with the Shift Coordinator and Medical Officer to review existing strategies and ensure assertive de-escalation/less restrictive strategies have been trialled or appropriately considered and excluded.

Risk mitigation strategies that must always be considered first include:

De-escalation skills and strategies	Increased nursing staff presence
Care/nursing/management/recovery plan and safety plan	Relocation of staff
Increased levels of observations	Use of special
Use of medication (current regimen /prn/ Stat orders)	Use of Comfort Room
Consultation with Specialist clinicians/Consultant Psychiatrist	Use of Seclusion Room

2. If the risk is unable to be adequately managed safely, then the risk is escalated to the Shift Coordinator who reviews the Consumer Safety Plan and Treatment, Support and Discharge Plan with the ward ANUM or Mental Health Afterhours CNS (MH AH CNS).
3. The ANUM or MH AH CNS escalate to the treating consultant, who reviews the patient with the treating team and documents the review in the patient's medical record. This review will include a plan for external security usage if required to maintain safety on the ward.

4. The Shift coordinator will identify the patient as a Patient of Concern (POC), Specifying "Aggression risk requiring external security" on i.Clinical Manager.
5. When there is one or more identified patients of concern requiring external security on the ward the ANUM or afterhours MH CNS will identify if 1 or 2 security are required to manage the level of risk on the ward.

Security Allocation

External security officers based on the ward are not explicitly allocated to the patient identified as a POC but are rather a resource to be utilised by the nursing staff to manage the overall increased risk present on the ward.

It is expected that if there is an individual patient identified as requiring the use of an external security, then only one officer would be provided. The risk posed by multiple POC being identified on the ward is to be reviewed and a decision made if the level of risk present on the ward is able to be adequately controlled by an individual security officer or if two security officers are required.

External Security is to be capped at 2. In the event that two external security officers are insufficient to manage the level of risk additional nursing staff may be utilised to support the ward.

In exceptional circumstances, eg. additional nursing staff unable to be facilitated, or if additional security personnel are required due to significantly elevated clinical risk, the treating team should discuss the level of risk and management plan with the Service Director and Head of Service. Any request for additional security beyond two personnel must be formally endorsed by the Service Director.

3.2 Contacting external Security Agencies.

1. The ANUM or if afterhours MH CNS will liaise directly with the external agencies providing as much information and notice as is clinically possible.
2. MH AH CNS provides swipe access, keys and duress pendants to external security contractors at the commencement of the shift.
3. The Shift Coordinator is responsible for providing the security officers with orientation, handover, direction and duties required whilst on the ward ensuring adherence to least restrictive practice principles.

3.3 Review of external security usage

1. ANUM or AH CNS are to identify security usage as part of the MH Huddle.
2. Patients who have required external security use are to be reviewed by the treating team, inclusive of the consultant prior to 1500hrs each day using template for Initiating or extending security use, ensuring that the review identifies if external security usage remains indicated.
3. The Nursing Coordinator Mental Health will review these daily, providing a report to Service Director Mental Health if external security is in use for a period exceeding 72hrs for an individual patient's risk.

4. Any cancellation of security will be communicated by the ANUM or AH CNS directly to the external contractors when the treating team decide that additional security is no longer required.

3.4 Process flowchart.

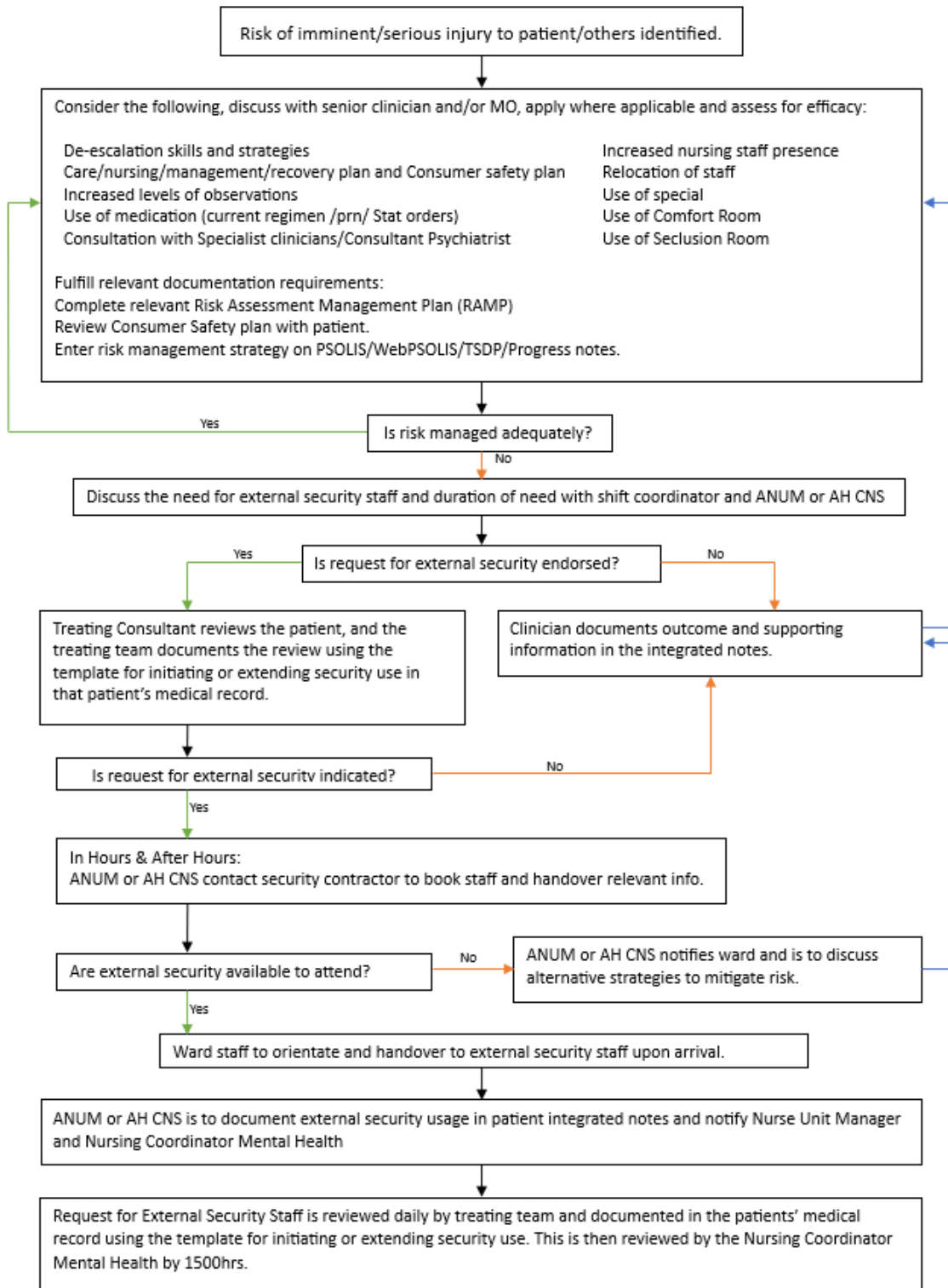


Figure 1

4. Roles and responsibilities

The following outlines the specific roles and responsibilities relevant to this procedure:

Role	Responsibility
Service Director Mental Health	- The SDMH has overall responsibility for ensuring the wellbeing of staff, patients and visitors and the financial management of external Security use at Armadale Mental Health Service. - The Service Director Mental Health will review the use of external security using the reports provided by the Nursing Coordinator Mental Health
HoD Psychiatry	The Head of Psychiatry has overall responsibility for patient safety ensuring that medical staff are given orientation and education on the use of this procedure and to ensure that services are delivered in accordance with this procedure.
Nursing Coordinator Mental Health	The Nursing Coordinator for Mental Health has overall responsibility for ensuring that inpatient services are delivered in accordance with this procedure
Nurse Unit Manager	To ensure the provision of orientation and education to relevant AKG clinicians and staff on the use of this procedure
Associate Nurse Unit Manager	To provide feedback to the Nurse Unit Manager and are required to work within this procedure.
Afterhours Mental health Clinical Nurse Specialist	To provide feedback to the Nurse Unit Manager and are required to work within this procedure.
AKG Staff	All AKG mental health staff are required to work within this procedure.

5. Compliance monitoring

Compliance against this procedure will be monitored by the [insert governing body].

Compliance monitoring activities relevant to this procedure include:

- CHoIRs
- Report generated by NC on continues security usage.
- Clinical incidents report

6. Policy context

The following documents are required to give effect to this procedure:

- *Mental Health Act 2014*
- *Occupational Safety and Health Act 1984*
- *Occupational Safety and Health Regulations 1996*
- DOH [Safety Planning for Mental Health Consumers Policy](#)
- DOH [Workplace Aggression and Violence Policy and Guidelines](#)

- DOH [State-wide Standardised Clinical Documentation SSCD for Mental Health Services OD 0526/14](#)
- DOH [Clinical Incident Management Policy MP 0122/19](#)
- Chief Psychiatrists: [Reporting Notifiable Incidents to the Chief Psychiatrist](#)
- East Metropolitan Health Service (EMHS) [Security Policy EMHS: 114](#)
- EMHS [Duress Alarm Systems EMHS: 15](#)
- EMHS [Prevention and Management of Workplace Aggression and Violence Policy EMHS: 127](#)
- EMHS [OSH: Incident and Hazard Reporting Policy EMHS: 06](#)

7. Supporting information

The following documents support the implementation of this procedure:

- [AKG Duress System Personal Leschen Unit Procedure](#)
- [AKG Code Black Response Plan](#)
- [AKG Mental Health Restraint Policy](#)
- [AKG Mental Health Seclusion Policy](#)
- [AKG Consumer Safety Planning Procedure](#)
- [AKG Safe and Supportive Observations Procedure](#)

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.01
- Standard 1 – Action 1.06
- Standard 1 – Action 1.30
- Standard 2 – Action 2.01
- Standard 2 – Action 2.10
- Standard 5 – Action 5.01
- Standard 5 – Action 5.03
- Standard 5 – Action 5.07
- Standard 5 – Action 5.10
- Standard 6 – Action 6.01
- Standard 6 – Action 6.03
- Standard 6 – Action 6.09
- Standard 6 – Action 6.11

9. Document contact

Enquiries relating to this procedure may be directed to:

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Department: Mental Health
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10. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3.0	22/08/2025	22/0-8/2028	Scheduled review

11. Authorisation

This procedure has been authorised and issued by: Service Director Mental Health

Authorised by	Dene Olive – A/Service Director Mental Health
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